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CRITICAL CARE

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MANAGEMENT OF ALCOHOL WITHDRAWAL

REVIEW BY (MAR 2024)

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Key Points

- All patients admitted to hospital should be asked about their alcohol consumption
- Quantifying alcohol consumption will help recognise the process of withdrawal and help prevent progression to more advanced stages.
- CIWA-Ar scale should be used to assess withdrawal
- Reducing dose regimens of chlordiazepoxide should be based on the cumulative total of a 24hr period of PRN dosing.

INTRODUCTION

Managing patients who are withdrawing from alcohol can pose a challenge to clinicians. Having system in place to assess and manage alcohol withdrawal allows clinicians to intervene in a timely fashion and provide appropriate pharmacotherapy to those patients who require it.

ASSESSMENT

The assessment tool used to gauge the severity of alcohol withdrawal is the **Clinical Institute Withdrawal Assessment – Alcohol (Revised)** or **CIWA-Ar** score. Shortened from the original CIWA score, the **CIWA-Ar** score measures **10** symptoms through a combination of observation and questioning. Each symptom is given a score.

SYMPTOM	SCORE RANGE
Nausea and vomiting	0-7
Tremor	0-7
Paroxysmal sweats	0-7
Anxiety	0-7
Agitation	0-7
Tactile disturbances	0-7
Auditory disturbances	0-7
Visual disturbances	0-7
Headache	0-7
Orientation and Clouding of Sensorium	0-4

MAX SCORE = 67

PROCESS

- Patients who begin showing signs of alcohol withdrawal are assessed using the **CIWA-Ar score** for 3 days - Day 0: from onset of symptoms to 7am the next morning
  Day 1: 8am to 7am
  Day 2: 8am to 7am

- Patients are treated with PRN doses of **Chlordiazepoxide** as follows:

CIWA ≤ 9: No dose of chlordiazepoxide. Repeat score in 4 hours (or sooner if needed)

CIWA 10-15: 20mg Chlordiazepoxide. Repeat score in 4 hours

CIWA 16-20: 20mg Chlordiazepoxide. Repeat score in 1 hour

CIWA ≥ 21: 40mg Chlordiazepoxide. Repeat score in 1 hour. **Urgent medical review.**

- Day 3: The patients **cumulative** PRN Chlordiazepoxide usage for **DAY 2 ONLY** is calculated and the patient is then placed on the corresponding reducing dose regimen as follows:

<40mg: No reducing regimen required

40-60mg: BLUE REGIMEN

2 days reducing from an initial dose of 10mg QDS

60-120mg: YELLOW REGIMEN

4 days reducing from an initial dose of 20mg QDS

140-180mg: ORANGE REGIMEN

6 days reducing from an initial dose of 30mg QDS

200-240mg: RED REGIMEN

8 days reducing from an initial dose of40mg QDS

- >240mg cumulative requirements – seek a senior review
- All the above regimens include a PRN prescription of 20mg in addition to the reducing dose
- Once a patient is on a reducing dose regimen CIWA scoring need only be carried out if the patient shows signs and symptoms of withdrawal. A score ≥ 10 should be treated with a PRN dose
- If CIWA score is consistently >15 and frequent PRN doses of Chlordiazepoxide are required a regimen choice review should be considered

CONCOMITANT MEDICATIONS

- All patients with a suspected Hx of alcohol abuse should be given prophylactic parenteral B Complex vitamins (Pabrinex) 1 pair IV or IM OD for 3-5 days (higher doses will be required if Wernicke's encephalopathy is suspected)
- PRN prescription for an anti-seizure medication should be written
- When higher doses of Chlordiazepoxide are being used Flumazenil should be considered if Benzodiazepine toxicity is suspected.

Alcohol Nursing Observation Chart 0

CIWA-Ar dosing.pdf